

GENDER AFFIRMING CARE — Clinical One-Pager

Adult outpatient GAHT · WPATH SOC8 (2022) & Endocrine Society (2017) · IM resident reference

1 · TERMINOLOGY & AFFIRMING INTAKE

Four independent axes: gender identity (internal) · gender expression (outward) · sex assigned at birth (AMAB/AFAB; DSD formerly "intersex") · sexual orientation (attraction).

Ask about	Patient-centered phrasing
Name	"What name do you go by?" — avoid <i>deadnaming</i>
Pronouns	"What pronouns do you use?" (he / she / they / ze)
Gender ID	"How would you describe your current gender identity?"
Sex @ birth	"What sex was assigned to you at birth?"
Organ inv.	"What organs do you have / have had removed?" — drives screening

If you **misgender**: Recognize → Apologize → Commit to change → Move on (briefly).

2 · EVALUATION — INFORMED CONSENT

Dx **gender dysphoria** (DSM-5) **can be made by a generalist**. Gender incongruence is **not** a mental illness. No mandatory MH gatekeeper to start hormones; SOC8 = **one** letter for surgery.

- **Capacity** — understands risks/benefits/reversibility
- **Contraindications** — e.g. estrogen-sensitive cancer, high VTE risk (estrogen)
- **Expectations** — changes, timeline, what's irreversible
- **Fertility counseling** — offer banking BEFORE starting

Options span: **GAHT, surgery, voice therapy, mental health** — guided by patient goals in an interdisciplinary medical home.

3 · FEMINIZING THERAPY

Two jobs: add estrogen + lower/block testosterone. **Goal estradiol 100–200 pg/mL; total T <50 ng/dL.**

Drug	Notes / side effects
17β-estradiol	PO / SL / patch / IM. VTE (patch = lowest risk); estrogen-dependent cancer = contra
Spironolactone	#1 anti-androgen. ↑K ⁺ , orthostasis, ↑urination, ↓libido
5α-reductase inh	Anti-androgen; usually <i>with</i> spiro
GnRH agonist	Potent suppression; cost; bone health

Timeline: ↓erections/libido + fat redistrib ~3 mo · breast growth 2–5 y (**irreversible**) · ↓body hair 6–12 mo (often incomplete).

Adapted from TeachIM.org "Gender Affirming Care" (Adams, Hedian, Lee) under CC BY-NC 4.0. Educational use only; not medical advice. Clinician-reviewed before teaching. Optimized June 2026.

4 · MASCULINIZING THERAPY

Testosterone (single agent): IM / SC / topical. **Goal trough total T 400–700 ng/dL.** Key risk: **erythrocytosis** (check Hct → r/o OSA; ↓dose / ↑freq / → topical).

Timeline: ↑acne/oil + ↑libido early · voice deepening (**irreversible**) · menses cessation ~70% (~30% persist) · ↑muscle + fat redistrib · clitoral enlargement (**irreversible**) · ↑facial/body hair + scalp loss.

5 · MONITORING

Levels + exam at **3 & 6 months, then annually.**

Regimen	Labs
Feminizing	Estradiol, total T, LFTs, lipids; +K ⁺ /Cr if on spironolactone
Masculinizing	Total T (± estradiol), Hct/CBC

6 · SAFETY & FAQ

DVT/PE on estradiol	Patient-centered; patch lowest-risk; transdermal + anticoag if continuing; stop only if symptomatic PE / instability
Polycythemia on T	r/o causes + OSA; ↓dose / ↑freq / → topical
CV risk	Mild ↑; USPSTF screening, modify risk factors
Cancer screening	By organs present , not gender identity
Inpatient / periop	Do NOT hold — except estradiol w/ VTE risk
Cost / access	Medicaid variable; Medicare covers; surgery often self-pay

2025 access: EO 14187 (Jan 2025), proposed CMS rules (Dec 2025), ACA EHB Final Rule (Jun 2025) mainly restrict care/coverage for <19. Adult standard unchanged — **verify current local law & payer policy.**